Supplemental material 1: Search Strategy

Table S1_1. Search strategy in PubMed and Medline.

Boolean Operator	Search strategy
	intervention [tiab] OR workshop [tiab] OR training [tiab] OR education [tiab] OR leadership* OR occupation* OR workplace OR organization* OR "Inservice training" [mh]
AND	
	team* [tiab] OR nurs* [tiab] OR *personnel* [tiab] OR employee* [tiab] OR staff [tiab] OR worker* [tiab] OR *professional* [tiab]
AND	
	"outpatient care" OR "outpatient nursing care" OR ambulant nursing* OR ambulatory care [mh] OR "home care"

Table S1_2. Search Strategy in APA PsycArticles.

Boolean Operator	Search strategy
	Intervention OR workshop OR training OR education OR leadership* OR occupation* OR workplace OR organization* OR “inservice training”
AND	
	team*nurs* OR *personnel* OR employee* OR staff OR worker* OR professional*
AND	
	“outpatient care” OR “outpatient nursing care” OR “ambulant nursing*” OR “ambulatory care” OR “home care”

Table S1_2. Search strategy in Web of Science.

Boolean Operator	Search strategy
	TS=(Intervention OR workshop OR training OR education OR leadership* OR occupation* OR workplace OR organization* OR "Inservice training")
AND	
	TS=(team* OR nurs* OR *personnel* OR employee* OR staff OR worker* OR *professional*)
AND	
	TS=("outpatient care" OR "outpatient nursing care" OR "ambulant nursing*" OR "ambulatory care" OR "home care")

For Peer Review

Supplemental material 2: Results from the Joanna Briggs Institute (JBI) Critical Appraisal Tools

Table S2_1. Quasi-experimental studies, single studies

JBI questions	Andersen et al., 2015	Coogle et al., 2007	Czuba et al., 2012	Fallahpour et al., 2020	Gerstel et al., 2012	Hartvigsen et al., 2005	Jellema et al. 2002	Leff et al., 2000	Olson et al., 2015	Olson et al., 2018	Petterson et al., 2006	Pohjonen & Ranta 2001	Rosswurm et al., 2003
Is it clear in the study what is the 'cause' and what is the 'effect' (i.e. there is no confusion about which variable comes first)?	Y	Y	U	Y	Y	Y	Y	U	Y	Y	Y	Y	Y
Were the participants included in any comparisons similar?	N/A	Y	N/A	Y	Y	Y	Y	Y	Y	Y	Y	U	Y
Were the participants included in any comparisons receiving similar treatment/care, other than the exposure or intervention of interest?	N/A	N/A	Y	N/A	Y	Y	N/A	N/A	N/A	N/A	N/A	Y	N/A
Was there a control group?	N	N	N	N	Y	Y	N	N	N	N	N	Y	N
Were there multiple measurements of the outcome both pre and post the intervention/exposure?	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y
Was follow up complete and if not, were differences between groups in terms of their follow up adequately described and analyzed?	N	Y	U	U	U	Y	Y	U	Y	U	N	Y	Y
Were the outcomes of participants included in any comparisons measured in the same way?	U	Y	Y	Y	Y	Y	Y	N/A	Y	Y	Y	Y	Y
Were outcomes measured in a reliable way?	N/A	Y	U	N/A	Y	N/A	N/A	N/A	N/A	N/A	N/A	N/A	N/A
Was appropriate statistical analysis used?	Y	U	Y	Y	Y	Y	Y	N/A	U	N/A	Y	Y	N/A

Note. Y: Yes, N: No, U: Unclear, N/A: not available

Table S2_2. Randomized controlled trials, single studies

JBI questions	Bagheri et al., 2021	Fujita et al., 2019	Fukui et al., 2019	Horneij et al., 2001	Olson et al., 2016	Roelofs et al., 2007
Was true randomization used for assignment of participants to treatment groups?	U	U	U	U	U	Y
Was allocation to treatment groups concealed?	N/A	N/A	N/A	N/A	N/A	N/A
Were treatment groups similar at the baseline?	U	Y	Y	Y	Y	Y
Were participants blind to treatment assignment?	N	N	N	N/A	N	N
Were those delivering treatment blind to treatment assignment?	N/A	N	N/A	N/A	N/A	N
Were outcomes assessors blind to treatment assignment?	U	N	N/A	N/A	N/A	N
Were treatment groups treated identically other than the intervention of interest?	Y	Y	Y	Y	Y	Y
Was follow up complete and if not, were differences between groups in terms of their follow up adequately described and analyzed?	Y	Y	Y	Y	U	Y
Were participants analyzed in the groups to which they were randomized?	U	U	U	U	Y	Y
Were outcomes measured in the same way for treatment groups?	Y	Y	Y	Y	Y	Y
Were outcomes measured in a reliable way?	N/A	Y	N/A	N/A	N/A	N/A
Was appropriate statistical analysis used?	Y	Y	Y	Y	Y	Y
Was the trial design appropriate, and any deviations from the standard RCT design (individual randomization, parallel groups) accounted for in the conduct and analysis of the trial?	Y	Y	Y	Y	Y	Y

Note. Y: Yes, N: No, U: Unclear, N/A: not available

Supplemental material 3: Extended Table of Evidence

Table S3_1 Extended description of the included studies

Reference	Intervention modalities (extended description)	Control group	Outcomes and measurements	Results
Andersen et al., 2015	Intervention: Organizational and technological measures to enhance perceived work demands and musculoskeletal health. (1) New work program: Specification and systematization of duties concerning patients' discharge from hospital and return to home, and overall functioning at home; implementation of related checklists. (2) New technological device: Personal digital assistant (PDA) including number and order of patients to visit, activities to be carried out, the amount of time allotted to each visit; medical literature, information and up-dates regarding patients, enables instant documentation and report writing. (3) Supplementary measures implemented at changeover to the two new procedures: 2-day training course, optional additional individual training. Duration total: 2 years Frequency: N/A	N/A	-Physical demands: visual analog scale 0-10 -Mental demands: visual analog scale 0-10 -Social demands: visual analog scale 0-10 -Emotional demands: visual analog scale 0-10 -Perceived general tension: visual analog scale 0-10 -Pain in shoulder-neck: visual analog scale 0-10 -Pain in low back regions: visual analog scale 0-10	→Physical demands, →Mental demands, →Social demands, →Emotional demands, →Perceived general tension, →Pain in shoulder-neck, →Pain in low back regions Sub-group low & moderate strain: →Physical demands, ↓Mental demands, ↓Social demands, ↓Emotional demands, →Perceived general tension, →Pain in shoulder-neck, →Pain in low back regions Sub-group high strain: →Physical demands, →Mental demands, →Social demands, →Emotional demands, →Perceived general tension, →Pain in shoulder-neck, →Pain in low back regions
Bagheri et al., 2021	Intervention: Slip resistance winter footwear to reduce fall risk during working hours Wearing slip resistance winter footwear; Sperry Women's Powder Valley boot (model STS97168), Windriver Men's Bivy boots (model number 5CPEWRF17-5251) Duration total: 8 weeks Frequency: N/A	Wearing winter footwear of their choice	-Slip rate (Self-developed question) -Fall rate (Self-developed question)	↓Slip rate in the intervention compared to the control group ↓Fall rate in the intervention compared to the control group

Note: N/A: not available; → no significant results; ↓ significant decrease; ↑ significant increase

For Peer Review

Table S3_1 Extended description of the included studies (continued)

Reference	Intervention modalities (extended description)	Control group	Outcomes and measurements	Results
Coogle et al., 2007	Intervention: The enhanced care assistant training (ECAT) curriculum to enhance job satisfaction and career commitment 4 modules (each with 1 7-hour training session); development and execution of personal action plans (PAP) (two goals based on the objectives of the module, with corresponding task and timelines; peer mentoring to share the progress of the PAP; when the develop in the next module the new PAP they had to report on the completion of the PAP of the module before) Topics: Module 1: Talking the talk: ways to smooth out problems without getting into more trouble – communication challenges; professionalism; working with difficult families. Module 2: Mind, body, & soul: challenges of caring for clients with cognitive, sexual and spiritual needs - aims at improving skills required to recognize and meet multiple needs of clients. Module 3: It's OK to become attached to your clients: dealing with loss, death and mourning – learn about multiple losses experienced by older people and acquired new strategies for helping others to cope with death. Module 4: Avoiding burnout: caring for others by caring for ourselves. Aims: recognize and avoid burnout or lessen its impact. Duration total: 1 year, 40 hours Frequency: four 7-hour training sessions, 12 hours self-study	N/A	**Job satisfaction:** -Minnesota Satisfaction Questionnaire (MSQ, 2 subscales: intrinsic job satisfaction, extrinsic job satisfaction) **Career commitment (CCM)**: -Career Commitment Measure (CCM, 3 subscales: career identity, career planning, career resilience) **Feedback on changes of work situation:** -5-point Likert scale influence of training on job retention -Abilities to perform job related tasks -Deal with responsibilities	**Job satisfaction:** →MSQ-total score, →Intrinsic job satisfaction, ↓Extrinsic job satisfaction **Career commitment:** →Total CCM, →Career identity, →Career planning, →Career resilience **Job satisfaction age groups:** -MSQ-total score: →18-39 years, ↓40-51years, →52+years -Intrinsic job satisfaction: ↑18-39 years, ↓40-51 years, →52+ years -Extrinsic job satisfaction: →18-39 years, ↓40-51 years, →52+ years **Career commitment age groups:** -Total CCM: →18-39 years, →40-51 years, →52+ years -Career identity: →18-39 years, →40-51 years, →52+ years -Career planning: →18-39 years, →40-51 years, →52+ years -Career resilience: →18-39 years, →40-51 years, →52+ years **Feedback on changes of work situation** (No statistical test; arrows indicate low, medium, high means): ↑Job retention, ↑Abilities to perform job related tasks, ↑Deal with responsibilities, →Stress related items

Note: N/A: not available; → no significant results; ↓ significant decrease; ↑ significant increase

For Peer Review

Table S3_1 Extended description of the included studies (continued)

Reference	Intervention modalities (extended description)	Control group	Outcomes and measurements	Results
Czuba et al., 2012	Intervention: Organizational level support program for improving work conditions (1) Providing cleaning supplies, gait belts (2) More information on their patients (i.e., care plans) (3) Testing the patient workload distribution system: each patient was categorized by their amount of physical effort required to care for the patient. The schedulers have to follow the scheduling rules of the workload distribution system, therefore no more than 50% of visits with patients categorized as 4 or 5 (highest level) in a day. Duration total: 3 weeks Frequency: N/A	N/A	-Perceived level of fatigue: perceived fatigue level at the end of the day (0–10)	↓Perceived level of fatigue (comparing the groups of <50% of visits with patients categorized as 4 or 5 in a day with >50% of those patients)
Fallahpour et al., 2020	Intervention: Dementia care education program -Knowledge translation: providing a shared understanding of Swedish national guidelines for care of persons with dementia and the content to the home care staff -Knowledge generation: to develop, implement, and evaluate a practice improvement; ten home-based improvement seminars (small groups) -Knowledge dissemination: to share experiences, knowledge and skills; one facility-wide day; one facility wide poster session -Two components: evidence-based care, person-centered care Duration total: 12 months Frequency: Seminar (4h) and workshop (4h)	N/A	**-Job strain:** Strain in Dementia Care Scale (SDCS); frustrated empathy, difficulty in understanding and interpreting, balancing competing needs, balancing emotional involvement, lack of recognition **-Organizational climate:** Creative Climate Questionnaire (CCQ; challenge, freedom, idea support, trust/openness, dynamism/liveliness, playfulness/humor, debates, risk-taking, idea time)	**Job strain** →Total strain SDCS, frustrated empathy, difficulty in understanding and interpreting, balancing emotional involvement, lack of recognition ↓job strain for the subscale 'balancing competing needs' **Organizational climate** →Total CCQ, challenge, freedom, idea support, trust/openness, dynamism/ liveliness, playfulness/humor, debates, risk-taking, idea time

Note: N/A: not available; → no significant results; ↓ significant decrease; ↑ significant increase

For Peer Review

Table S3_1 Extended description of the included studies (continued)

Reference	Intervention modalities (extended description)	Control group	Outcomes and measurements	Results
Fukui et al., 2019	Intervention: Multidisciplinary end-of-life educational intervention program (1) Educational booklet: information on collaboration, timing and style of information sharing, characteristics and symptoms of patients (eight phases of end-of-life care). Covered in 3 Topics: (a) Mutual respect as professionals who have different expertise, (b) Understanding of professionals' roles, (c) Sharing of information in a timely manner to support end-of-life patients appropriately (2) Lecture and group work session: (a) Lecture (30 minute): explanation about the aim and content of the booklet; (b) Workshop (150 minutes): interdisciplinary group work, discussion and role-play on sharing information (3) Participants should implement the booklet contents during the 7 months intervention phase Duration total: 7 months Frequency: two 3-hour programs	Usual care	**-Confidence in inter-professional collaboration:** Face-to-Face Cooperative Confidence Questionnaire (FCCQ; subscales: I can smoothly communicate with medical and social care professionals in other facilities; I understand other occupational roles of people working in the community; I know the face, name, and characteristics of people associated with home care in the community; I have an opportunity to discuss matters with other health care workers in the community; I am connected to community care networks; I specifically understand community resources; a good network is maintained between the hospital and community) **-Job satisfaction:** Minnesota Satisfaction Questionnaire (MSQ, subscales: intrinsic satisfaction, extrinsic satisfaction, general satisfaction)	**Confidence in inter-professional collaboration**: ↑I can smoothly communicate with medical and social care professionals in other facilities; ↑I understand other occupational roles of people working in the community; ↑I know the face, name, and characteristics of people associated with home care in the community; ↑I have an opportunity to discuss matters with other health care workers in the community; ↑I am connected to community care networks; ↑I specifically understand community resources; ↑A good network is maintained between the hospital and community **Job satisfaction:** →Intrinsic satisfaction (extrinsic satisfaction and general satisfaction N/A)
Fujita et al., 2019			-**Understanding own and other professional roles on end-of-life care:** self-developed tool 6 items (0-10) -**Mutual respect among team members:** self-developed tool 5 items (0-10) -**Confidence in collaboration among health and welfare professionals:** self-developed tool 1 item (0-10)	**Understanding own and other professional roles on end-of-life care:** →Homecare nurse, ↑Care manager, ↑Head care worker **Mutual respect among team members:** →Homecare nurse, →Care manager, ↑Head care worker **-Confidence in collaboration among health and welfare professionals:** →Homecare nurse, ↑Care manager, ↑Head care worker

Note: N/A: not available; → no significant results; ↓ significant decrease; ↑ significant increase

For Peer Review

Table S3_1 Extended description of the included studies (continued)

Reference	Intervention modalities (extended description)	Control group	Outcomes and measurements	Results
Gerstel et al., 2013	Intervention: Physical activity and nutrition program **(1) Mobility program** (a) Provision of bikes for the home visits (regular, electric, foldable) (b) Instructions to walk short distances (< 300 m), cycle for medium distances (< 2 km), use of public transportation or a car-sharing system for longer distances or in cases of disability or bad weather. **(2) Educational program** (a) 6-week cognitive behavioral education program on physical exercise and adequate nutrition focusing on participants' own experience-centered approach and long-term behavioral changes. (b) Further components: Trigger recognition, diet analysis, self-control and awareness learning and tools for maintaining new habits Duration total: 12 months Frequency: N/A	No intervention, single workshop at the end of the study	**Anthropometric parameters:** -Body weight -BMI -Body fat -Waist circumference **Health-related quality of life:** -Short-Form Health Survey (SF-36) **-Dietary behavior** -Food Frequency Questionnaire (FFQ; rest metabolism, total caloric intake, protein intake, lipid intake, carbohydrate intake) **-Physical activity behavior:** -Physical Activity Frequency Questionnaire (PAFQ; low-intensity physical activity, moderate physical activity, intense physical activity) **-Blood parameters:** -Systolic/diastolic blood pressure -Fasting glucose -Total cholesterol -Low-density lipoprotein cholesterol -High-density lipoprotein cholesterol -Triglycerides	Within group changes for IG, at 6 months: **Anthropometric parameters:** ↓Body weight, ↓BMI, ↓Body fat, →Waist circumference **Blood parameters:** ↓Systolic blood pressure, →Diastolic blood pressure, →Fasting glucose, ↓Total cholesterol, ↓Low density lipoprotein cholesterol, ↑High-density lipoprotein cholesterol, →Triglycerides **Health-related quality of life:** →Total score **Dietary behavior:** →Rest metabolism, ↓Total caloric intake, →Protein intake, →Lipid intake, →Carbohydrate intake **-Physical activity behavior:** →Low-intensity physical activity, →Moderate physical activity, →Intense physical activity Within group changes for IG, at 12 months: **Anthropometric parameters:** ↓Body weight, ↓BMI, ↓Body fat, waist circumference **Blood parameters:** ↓Systolic blood pressure, →Diastolic blood pressure, ↓Fasting glucose, ↓Total cholesterol, ↓Low density lipoprotein cholesterol, ↑High-density lipoprotein cholesterol, →Triglycerides **Health-related quality of life:** →Total score **Dietary behavior:** →Rest metabolism, ↓Total caloric intake, →Protein intake, →Lipid intake, →Carbohydrate intake **Physical activity behavior:** ↑Low-intensity physical activity, →Moderate physical activity, →Intense physical activity

Note: BMI: Body Mass Index, IG: Intervention group, N/A: Not available; → no significant results; ↓ significant decrease; ↑ significant increase

For Peer Review

Table S3_1 Extended description of the included studies (continued)

Reference	Intervention modalities (extended description)	Control group	Outcomes and measurements	Results
Hartvigsen et al., 2005	Intervention: Intensive educational and low-tech ergonomic program Small Groups of 8-12; one person was educated from a physiotherapist as instructor for each group in rational body mechanics and lifting techniques according to the Bobath principle, usage of low-tech ergonomic aids such as plastic sheets Duration total: 2 years Frequency: 1h per week: educated instructor and the small groups; 2h four times during the first seven months; 1h every 6 weeks meetings between all group instructors	3-hour instruction in lifting technique, no low-tech ergonomic devices were available	Low back pain (LBP): Items from Standardised Nordic Questionnaire: number of days with LBP (past year), number of episodes of LBP (past year), seeking of treatment (past year)	**Low back pain** →Number of days with LBP (past year), →Number of episodes of LBP (past year), →Seeking of treatment (past year)
Horneij et al., 2001	**(Arm 1) Individual physical training program (IT):** All participants received an individually designed training program based on the results of a physical examination. Implementation in daily life was discussed, individual goals were formulated; participants make diary entries; the program and the diary were followed up after about 1 and 2 weeks, after 3 months, in connection physical examination after 6 months. **(Arm2) Stress management program in groups (SM):** To identify and reach goals and strategies for perceived stress induced by lack of social support, low decision latitude/work control, and perceived high psychological workload; 7 1.5-hour sessions in 7 weeks, theory and practice; stress-reduction goal setting (individual; entire workplace); two follow-ups after about 3 and 6 months. Interactive discussion about experience of stress in general and at work, and how to handle these problems Duration total: 6 months Frequency: Arm 1: N/A Arm 2: 7 sessions each 1.5 hours	No intervention	**Musculoskeletal disorders:** -Neck, shoulder and back symptoms: Nordic Musculoskeletal Questionnaire (NMQ) **Pain:** -Perception of pain: self-developed tool 5 scores -Perceived interference with work and/or leisure activities: neck/shoulders pain (previous month), low back pain (previous month) -Pain-drawing: sum of areas **Perceived physical exertion at work:** -Borg scale **Perceived work-related psychosocial factors:** -Questionnaire by Rubenowitz (subscales: influence on and control over work, supervisor climate, stimulation from the work itself, relations with fellow workers, psychological workload) **Physical activities or fitness training:** -Extent of physical activities/fitness training (previous six months) -Perceived amount of training (at month 12, compared to previously) **Applied relaxation and home exercise:** -Reported at 18-month follow up (N/A)	→Musculoskeletal disorders (between all groups at all times) ↓Perceived interference with work and/or leisure activities due to discomfort in the low back (IT vs. CG, at 12months) →Perceived physical exertion at work (between all groups at all times) **Perceived work-related psychosocial factors:** ↓Supervisor climate (SM group compared with IT group and CG, month 18) ↓Stimulation from the work itself (SM group compared with IT group, month 18) **Physical activities or fitness training:** →Extent of physical activities/fitness training (between all groups at all times) ↑Perceived amount of training (SM compared to CG, at 18 months) →Applied relaxation and home exercises (between all groups)

Note: N/A: Not available, CG: Control group; → no significant results; ↓ significant decrease; ↑ significant increase

For Peer Review

Table S3_1 Extended description of the included studies (continued)

Reference	Intervention modalities (extended description)	Control group	Outcomes and measurements	Results
Jellema et al., 2002	Intervention: Wearing lumbar support (the Lumboloc and the Lumbotrain Lady) to reduce low back pain -Phase I: 1 week, participants wore the lumbar support every day -Phase II: 6 Months, wear the support only on those working days when they experienced LBP Duration total: 6 months 1 week Frequency: N/A	N/A	-Compliance: Percentage of subjects who used the lumbar support at work for ≥80% of the weeks in which they reported LBP at work. -Benefit from lumbar support (0–10 scale) -Pain intensity (0-10 scale) -Disability due to LBP (0-100 scale)	-61% used the lumbar support at work for ≥80% of the weeks, 81% of people with LBP at work used the lumbar support, 23% used the lumbar support at home -Participants scored the benefit they experienced as 7 -44.6% decrease in pain intensity -44.3% decrease in the degree of disability
Leff et al., 2000	Intervention: Prevention program of back and shoulder injuries **(1) Short term:** a) Train new nursing assistants in body mechanics during the first two weeks on the job. b) Annual body mechanics training for all nursing assistants, c) Patient-specific training for all involved in care of patients with special needs. d) Train supervisor in how to evaluate back safety when doing supervisory visits. e) Risk management forms to identify and follow up on hazardous situations. f) Providing transfer (gait) belts. g) Schedule maximum seven visits per day **(2) Mid-term:** a) Identify heavy-lift patients. Limit number of or alternate heavy-lift patients with lighter ones. b) Use videos for staff training in back safety. **(3) Long term:** Conduct postoffer, prehire physical therapy screening Duration: N/A Frequency: N/A	N/A	**Employee back and shoulder injuries:** -Number of employees injured (from 1996-1999)	Employee back and shoulder injuries (no statistical tests): ↓Number of employees injured

Note: N/A: not available; → no significant results; ↓ significant decrease; ↑ significant increase

For Peer Review

Table S3_1 Extended description of the included studies (continued)

Reference	Intervention modalities (extended description)	Control group	Outcomes and measurements	Results
Olson et al., 2015 (Pilot - COMPASS)	Intervention: Community of practice and safety support (COMPASS; Community of Practice = peer-to peer collaborative network) (1) Team-building workshop: get to know each other, learn the COMPASS meeting process. (2) Meetings: scripted peer-led approach, following a standardized format including education and social support: a) Work-life check-in (individually rate work and life status in the group and talk about it); b) Follow-up on goals from the prior meeting; c) Workbook lesson on health promotion and occupational safety topics; d) Set specific "take home goals" related to the topic (individual and team goals; e) Healthy food break; f) Work-life support: each team member writes a problem/issue on the board and rate its urgency, team brainstorms potential solutions, the person with the problem to selects a solution, and form an action plan; g) Reflection on the session; session topics: fruits and vegetables, back to healthy postures, functional fitness, take a load off with tools, communicating for hazard correction, mental health Duration total: 6 months Frequency: 4-hour team-building workshop, 6 monthly 2-hour meetings	N/A	**Well-being:** -Satisfaction with life: 5 Items 5-point Likert scale (N/A) -Negative and positive affect: Positive and negative affect schedule (past month) -Depressive symptoms: center for epidemiologic studies depression five-item measure (past week) -Mental and physical health: 2-item short form health survey, mental and physical health subscales **Psychosocial and stress** -Community of practice: 12-item experienced community of practice measure -Team cohesion: Six-Item Scale from Chin et al. -Frequency of interpersonal conflict with consumer-employers: 10-items on the basis of the Wright et al. measure -Work-family and family–work conflict: Five-Item Netemeyer et al. scales -Perceived stress: items from the Perceived Stress Scale -Job skill discretion, decision authority, psychological job demands: items from Job Content Questionnaire **Health behavior** -Fruit and vegetable consumption: National Cancer Institute (NCI) fruit and vegetable screener -Fat consumption: NCI fat screener -High-fat and high-sugar food consumption: questions about frequency of consumption sugar drinks, sugar snacks, fast food and bringing meals from home -Frequency of moderate to intense physical activity: healthy physical activity scale -Sleep quality: Pittsburgh Sleep Quality Index -Safety compliance: safe behavior, Three-Item Safety Compliance Scale; safety actions, Four-Item Scale, created for the study	**Well-being:** ↑Satisfaction with life, ↓Negative affect, →Positive affect, →Depressive symptoms, →Mental health, →Physical health **Psychosocial and stress:** →Community of practice, ↑Team cohesion, ↓Interpersonal conflict, →Work-family conflict, →Family–work conflict, →Perceived stress, →Job skill discretion, →Decision authority, →Psychological job demands **Health behavior** ↑Fruit and vegetable consumption, →Percentage of calories from fat, →Sugary snacks, →Sugar drinks, →Fast food, ↑Meals from home, →Physical activity, →Sleep quality, →Safety compliance **Musculoskeletal pain and discomfort** →Pain, →Pain severity, →Functional impairment, →Occupational Fatigue **Physical measures** →Blood pressure, →Cholesterol, ↑Triglycerides, →Glucose, →Body fat, →BMI, →Waist−hip ratio, →Grip strength, →Hamstring flexibility, ↑Walk test

Note: N/A: not available; → no significant results; ↓ significant decrease; ↑ significant increase

For Peer Review

Table S3_1 Extended description of the included studies (continued)

Reference	Intervention modalities (extended description)	Control group	Outcomes and measurements	Results
Olson et al., 2015 (Pilot - COMPASS) (continued)			**Musculoskeletal pain and discomfort** -Prevalence of musculoskeletal pain and discomfort in neck/shoulders, forearms/wrist, low back, lower extremities, 5-point Likert scale -Pain severity during the past 7 days 0-10 -Functional impairment over the past 7 days for nine common activities of daily living -Occupational fatigue: Swedish Occupational Fatigue Inventory physical discomfort and lack of energy subscales **Physical measures:** blood pressure, cholesterol, triglycerides, body fat, BMI, glucose, neck, waist, and hip circumference **Fitness:** grip strength, hamstring flexibility, 6-minute walk test	
Olson et al., 2016 (COMPASS Program)	Intervention: Community of practice and safety support (COMPASS; Community of Practice = peer-to peer collaborative network) Extension to the pilot program: -Peer leaders received manuals with additional instructions; brief facilitator training at t0 and t1 -Two workbooks for the monthly 2-hour meetings: (1) Workbook 1 (month 1-6): developed and revised on the basis of the pilot study (topics remained the same), (2) Workbook 2 (month 7-12): not fully scripted lessons, teams choose a schedule of readings from a menu of options; topics: sustaining our team, body mechanics or helping consumer–employers obtain tools, healthy eating on a budget or rethink your drink, home safety or injury slip ups, fitness walking or exercise fun, healthy sleep or coping with stress or hand hygiene Duration total: 12 months Frequency: N/A	Receiving health feedback (from baseline testing), access to the usual resources provided	**Experience of community practice:** N/A **Well-being:** 12-item Short Form Health Survey version 2.0 (mental and physical) **Safety behavior:** 5 questions about the frequency of specific actions (past 6 months) **Health behavior** -Daily fruit and vegetable consumption: National Cancer Institute fruit and vegetable screener -Frequency of consumption of sugar drinks, sugar snacks, fast food and bringing meals from home (N/A) -Physical activity behavior: healthy physical activity scale -Sleep quality: one item of Pittsburgh Sleep quality index **Psychosocial experiences and stress** -Interpersonal conflict with patients (N/A) -Perceived stress: Perceived Stress Scale **Musculoskeletal pain** -Adapted version of Nordic Questionnaire	**↑Experience of community practice** (month 6 and 12) **Well-being:** →Total, →Mental, →Physical **Safety behavior:** ↑Talked with client about improving unsafe conditions (month 12), ↑Corrected slip, trip, or fall hazards (month 12), →Corrected other hazards, ↑Used new tool or techniques for moving objects or clients (month 6), ↑Used new tools or techniques for housecleaning (month 6 and 12) **Health behavior:** ↑Fruit and vegetable consumption (month 12), →Sugar drinks, →Sugar snacks, →Fast food →Physical activity, ↓Meals brought from home (at month 12) **Psychosocial experiences and stress:** →Interpersonal conflict with patients, →Perceived stress

Note: N/A: not available; → no significant results; ↓ significant decrease; ↑ significant increase

For Peer Review

Table S3_1 Extended description of the included studies (continued)

Reference	Intervention modalities (extended description)	Control group	Outcomes and measurements	Results
Olson et al., 2016 (COMPASS Program) (continued)			**Functional impairment** -Activities of daily living (N/A) -Work-related injuries (N/A) -Lost work time attributable to illness or injury (N/A) **Physical measures** Blood pressure, cholesterol, triglycerides, fasten glucose, BMI **Fitness:** grip strength, hamstring flexibility, 6-minute walk test -Patient satisfaction: Home Care Client Satisfaction Inventory -Conflict with their home care worker: adapted interpersonal conflict scale -Patients' reports of HCWs' safety Actions: adaptations of the 5 safety behavior items from the HCW survey	**Functional impairment:** →Activities of daily living, ↓Work-related injuries, →Lost work time **Physical measures:** →Blood pressure, →Cholesterol, ↑HDL, →Triglycerides, →Fasten glucose, →BMI **Fitness measures:** ↑Grip strength, →hamstring flexibility, →6-minute walk test
Olson et al., 2018 Adapted COMPASS Program	Intervention: The Community of Practice and Safety Support (COMPASS) Adaptation Adjustments included: (1) Meetings: groups met every other week instead of once a month, reduction of meetings; (2) Content: only topics from guidebook one, removing the meal served during breaks; (3) Peer-leader approach: trainers support each group meeting as a "guide on the side", rotating volunteer peer-leaders at meeting two; (4) replacing some original group and individual goals to attend related OHCC trainings with new goals focused on workers making targeted work-environment and behavior Changes Duration: 12 months Frequency: N/A	N/A	**Experience of community practice:** N/A **Well-being:** 12-item Short Form Health Survey version 2.0 (mental and physical) **Safety behavior:** 5 questions about the frequency of specific actions (past 6 months) **Health behavior** -Daily fruit and vegetable consumption: National Cancer Institute fruit and vegetable screener -Frequency of consumption of sugar drinks, sugar snacks, fast food and bringing meals from home (N/A) -Physical activity behavior: Healthy physical activity scale	**↑Experience of community practice** **Well-being:** →total, →mental, →physical **Safety behavior:** →Talked with client about improving unsafe conditions, → Corrected slip, trip, or fall hazards, →Corrected other hazards, ↑Used new tool or techniques for moving objects or clients, →Used new tools or techniques for housecleaning **Health behavior:** ↑Fruit and vegetable consumption, →sugar drinks, →sugar snacks, →fast food, ↑Physical activity, ↑ Meals brought from home

Note: N/A: not available; → no significant results; ↓ significant decrease; ↑ significant increase

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Table S3_1 Extended description of the included studies (continued)

Reference	Intervention modalities (extended description)	Control group	Outcomes and measurements	Results
Petterson et al., 2006	Intervention: An individual, organizational, and worksite level program to promote good working conditions (a) Competence program: 60 hours: 1 day a week for 10 weeks; learning sessions on quality of care and ethics and work quality; training on self-competencies and leadership competencies; participants were qualified to become ''innovation leaders''. (b)Worksite competence circles: the circles aimed at initiating a learning organization; in each circle, 8-12 persons were trained for 5 days (30 hours) during a 5-week period by two innovation leaders (c) Local worksite projects: aims at starting worksite improvements projects adjusted to local needs. Project plans were elaborated jointly by the participating employees. Local reference groups, consisting of the unit manager, the project co-ordinator, the innovation leaders, and representatives for registered nurses and physiotherapists, supported the projects on the worksite level. The started projects referred to issues such as introduction of new employees, quality development, the meaning of quality to employees, patients and patients' relatives, routines for staff meetings and communication, nursing routines, and communication between day and night personnel Duration total: 18 months Frequency: 60 hours: 1 day a week for 10 weeks, 30 hours: 5 days for 5 weeks	N/A	**Workload:** -Workload and work demands: N/A -Change in workload: N/A -Physical workload: N/A **Staff resources:** -Change in staff resources to handle work: N/A -Support from the management: N/A -Support from workmates: N/A -Support from clients and their relatives: N/A -Job influence and control: N/A -Learning and development: N/A -Job satisfaction: N/A **Health and well-being:** -Psychosomatic symptoms: N/A -Musculoskeletal symptoms: N/A -Stress symptoms: N/A -Global health: single question: N/A -Exhaustion: N/A -Well-being: shortened version of General Health Questionnaire **Health –resources:** -Self-esteem: N/A -Mastery: N/A -Coping: coping index	No intervention effects over times were tested for home care worker separately; results reflect an increase or decrease of mean values at t0 compared to t1 **Workload:** ↑Workload and work demands, ↓Change in workload, ↓Physical workload **Staff resources:** ↑Change in staff resources to handle work, ↑Support from the management, ↑Support from workmates, ↑Support from clients and their relatives, ↑Job influence and control, ↓Learning and development, ↑Job satisfaction **Health and well-being:** ↑Psychosomatic symptoms, ↑Musculoskeletal symptoms, ↑Stress symptoms, ↓Exhaustion, ↓Well-being, ↑Global health **Health –resources:** ↑Self-esteem, ↓Mastery, ↑Coping

Note: N/A: not available; → no significant results; ↓ significant decrease; ↑ significant increase

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Table S3_1 Extended description of the included studies (continued)

Reference	Intervention modalities (extended description)	Control group	Outcomes and measurements	Results
Pohjonen & Ranta, 2001	Intervention: Worksite physical exercise intervention (1) 2h orientation and motivation session on the topic of physical fitness in physically demanding jobs, (2) 2 lectures on leisure-time physical activity, body weight (2h each); (3) Individual feedback and personal counseling due to the baseline fitness test; (4) Supervised exercise, two groups according to initial fitness level, 1h aerobic training, 1h strength training Duration total: 9 months Frequency: 1h twice a week	N/A	**Body constitution:** -Body weight and body fat: anthropometric measurements **Physical fitness:** -Dynamic muscular endurance: sit-up test, squatting -Maximal isometric strength of body extension: isometric trunk extension -Flexibility: sit-and-reach test -Maximal oxygen consumption (VO_{2max}): bicycle ergometer using an automatic gas exchange analyzer **Physical workload:** -heart rate during shift (telemetric sport tester system) **Perceived health status:** -Single item (1-5) **Work ability:** -Work Ability Index -Single item current work ability **Leisure-time physical activity:** -3 items from Mini-Finland Health Survey	**1-year follow-up:** **Body constitution**: ↓Body weight, ↓Body fat **Physical Fitness:** ↑Sit-up test, ↑Squatting, ↑Isometric trunk extension, ↑Flexibility, ↑VO_{2max} **→Physical workload** ↑Perceived health status **Work ability**: →Work Ability Index, ↑Current work ability **Leisure-time physical activity** (N/A) **5-year follow-up:** **Body constitution:** →Body weight, ↓Body fat **Physical Fitness:** ↑Sit-up test, →Squatting, →Isometric trunk extension, →Flexibility, →VO_{2max} →Physical workload ↑Perceived health status **Work ability:** →Work Ability Index, ↑Current work ability →Leisure-time physical activity
Roelofs et al., 2007	Intervention: Lumbar support belt Wear lumbar supports on working days on which they had or expected they might develop low back pain; participants could select 1 of 4 types of lumbar supports. short refresher course on healthy working methods Duration total: 12 months Frequency: N/A	N/A	**Frequency of low back pain:** Number of days with low back pain per month **Sick leave:** -Number of days of sick leave (1 year) -Number of days of sick leave due to low back pain (1 year) **Severity of low back pain**: rated on a scale 0-10 (previous week) **Functional status:** Quebec Back Pain Disability Scale	↓Frequency of Low back pain **Sick leave:** →Sick leave, ↓Sick leave due to low back pain ↓Severity of low back pain **Functional status:** ↓Disability

Note: N/A: not available; → no significant results; ↓ significant decrease; ↑ significant increase

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Table S3_1 Extended description of the included studies (continued)

Reference	Intervention modalities (extended description)	Control group	Outcomes and measurements	Results
Rosswurm et al., 2003	Intervention: Psychoeducational geriatric care program (1) Self-care, (2) Community resources and care-provision decision making, (3) Elder abuse, (4) Coping with loss and dying, (5) Aging and communication, memory, mobility, pain, nutrition, skin, bowel and bladder elimination Duration total: 2 weeks, 12 hours Frequency: N/A	N/A	-Resourcefulness: Self-Control Schedule (SCS) -Knowledge: self-developed 13-item geriatric care knowledge test -Competence: self-developed question (scale 1-10)	Resourcefulness: ↑Home care aides, →Home care nurses Knowledge: ↑Home care aides, ↑Home care nurses Competence: ↑Home care aides, ↑Home care nurses

Note: N/A: not available; → no significant results; ↓ significant decrease; ↑ significant increase

For Peer Review

Supplemental material 4: Information about excluded reports

1. Wrong intervention (n=22)

No intervention conducted/described (n=13)
Allen, J. W. (2016). Transitioning the RN to Ambulatory Care: An Investment in Orientation. *Nursing administration quarterly*, *40*(2), 115–121. https://doi.org/10.1097/NAQ.0000000000000151
Andersen, G. R. & Westgaard, R. H. (2013). Understanding significant processes during work environment interventions to alleviate time pressure and associated sick leave of home care workers--a case study. *BMC health services research*, *13*, Article 477. https://doi.org/10.1186/1472-6963-13-477
Andersen, I., Borritz, M., Christensen, K. B. & Diderichsen, F. (2010). Changing job-related burnout after intervention--a quasi-experimental study in six human service organizations. *Journal of occupational and environmental medicine*, *52*(3), 318–323. https://doi.org/10.1097/JOM.0b013e3181d1cd87
Beauvais, A. & Frost, L. (2014). Saving our backs: safe patient handling and mobility for home care. *Home healthcare nurse*, *32*(7), 430–436. https://doi.org/10.1097/NHH.0000000000000110
Burson, R. & Moran, K. J. (2015). Empowerment and engagement. *Home healthcare now*, *33*(1), 49–50. https://doi.org/10.1097/NHH.0000000000000178
Distasio, C. A. (2000). Violence against home care providers--stop it before it starts. *Caring: National Association for Home Care magazine*, *19*(10), 14–18.
Flannery, M. (2011). Creating and sustaining an effective coaching culture in home care: one organization's performance improvement related to aides and aide retention. *Home healthcare nurse*, *29*(5), 275–281. https://doi.org/10.1097/NHH.0b013e3182173a1d
Goodwin Gerberich, S. (2019). Verbal abuse against home care aides: another shot across the bow in violence against health care and other workers. *Occupational and environmental medicine*, *76*(9), 593–594. https://doi.org/10.1136/oemed-2019-105925
Logan-Mullings, G. & Ruel, S. R. (2014). It takes a team. *Home healthcare nurse*, *32*(6), 381. https://doi.org/10.1097/NHH.0000000000000095
Marrelli, T. & Narayan, M. C. (2013). Self-care, health models, and an international home care nurses organization update. *Home healthcare nurse*, *31*(9), 457–459. https://doi.org/10.1097/NHH.0b013e3182a897ce
Schouteten, R. (2004). Group work in a Dutch home care organization: does it improve the quality of working life?. *The International journal of health planning and management*, *19*(2), 179–194. https://doi.org/10.1002/hpm.752
Smith, K. (2012). Support in tough situations. *Nursing times, 108*(22-23), Article 27.
Yarkony, L. (2012). Respect and recognition: the keys to keeping boomers on the job. *Caring : National Association for Home Care magazine*, *31*(1), 30–34.

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Intervention focus not on occupational health, safety, and well-being (n=8)

Cotter, J. J., Welleford, E. A., Vesley-Massey, K. & Thurston, M. O. (2003). Collaborative community-based research and innovation. *Family & community health*, *26*(4), 327–337. https://doi.org/10.1097/00003727-200310000-00009

Elliott, B. & Dillon, C. A. (2012). Online learning: an innovative solution to meeting the challenges of staff education. *Journal for nurses in staff development : JNSD: official journal of the National Nursing Staff Development Organization*, *28*(6), 285–287. https://doi.org/10.1097/NND.0b013e31827259de

Fallahpour, M., Borell, L., Sandberg, L. & Boström, A. M. (2020). Dementia Care Education Targeting Job Strain and Organizational Climate Among Dementia Care Specialists in Swedish Home Care Services. *Journal of multidisciplinary healthcare*, *13*, 85–97. https://doi.org/10.2147/JMDH.S214378

Jantunen, S., Piippo, J., Surakka, J., Sinervo, T., Ruotsalainen, S. & Burström, T. (2020). Self-Organizing Teams in Elderly Care in Finland: Experiences and Opportunities. *Creative nursing*, *26*(1), 37–42. https://doi.org/10.1891/1078-4535.26.1.37

Kreiser, A. L., Adamski, T. & Gallagher, B. (2010). Building a peer mentor home health aide program: implications for home health aide retention. *Home healthcare nurse*, *28*(8), 480–486. https://doi.org/10.1097/NHH.0b013e3181ed7456

Mager, D.R. & Lange, J. W. (2016). The ELDER project: Evaluating end-of-life knowledge among health care providers. *Journal of Hospice & Palliative Nursing*, *18*(1), 22-28. https://doi.org/10.1097/NJH.0000000000000199

Redworth, F., Atkin, J. & Jones-Tatum, L. (2001). District nurses and home carers 3: project evaluation. *British journal of community nursing*, *6*(2), 74–79. https://doi.org/10.12968/bjcn.2001.6.2.74

Thulesius, H., Petersson, C., Petersson, K. & Håkansson, A. (2002). Learner-centred education in end-of-life care improved well being in home care staff: a prospective controlled study. *Palliative medicine*, *16*(4), 347–354. https://doi.org/10.1191/0269216302pm568oa

Intervention developed for clients (n=1)

Muramatsu, N., Yin, L. & Lin, T. T. (2017). Building Health Promotion into the Job of Home Care Aides: Transformation of the Workplace Health Environment. *International journal of environmental research and public health*, *14*(4), 384. https://doi.org/10.3390/ijerph14040384

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2. No available data (n=11)

Results are not presented separately for home care workers (n=11)
Blake, H., Zhou, D. & Batt, M. E. (2013). Five-year workplace wellness intervention in the NHS. *Perspectives in public health*, *133*(5), 262–271. https://doi.org/10.1177/1757913913489611
Christensen, J. R., Pajevic, M., Ilvig, P. M., Søgaard, K. & Jessen-Winge, C. (2019). Why people engage in a weight loss intervention at their workplace - a stratified case study. *BMC public health*, *19*(1), -article 20. https://doi.org/10.1186/s12889-018-6346-0
Hardman Smith, L., Aust, B. & Flyvholm, M. A. (2015). Exploring Environment-Intervention Fit: A Study of a Work Environment Intervention Program for the Care Sector. *The Scientific World Journal*, Article 272347. https://doi.org/10.1155/2015/272347
Johnsson, C., Carlsson, R. & Lagerström, M. (2002). Evaluation of training in patient handling and moving skills among hospital and home care personnel. *Ergonomics*, *45*(12), 850–865. https://doi.org/10.1080/00140130210160920
Lange, J. W., Mager, D. R. & Andrews, N. (2013). The ELDER expansion project: building cultural competence among long term home care workers. *Applied nursing research : ANR*, *26*(2), 58–62. https://doi.org/10.1016/j.apnr.2012.10.003
Mager, D. R. & Lange, J. (2014). Teambuilding across healthcare professions: the ELDER project. *Applied nursing research*, *27*(2), 141–143. https://doi.org/10.1016/j.apnr.2013.06.00
Ng, S. M. (2014). Is brief daily body–mind–spirit practice desirable for staff who provide services for elderly people? Two pilot studies with care and professional workers. *Asia Pacific Journal of Social Work and Development*, *24*(4), 227-237. https://doi.org/10.1080/02185385.2013.843471
Oostrom, J. K. & van Mierlo, H. (2008). An evaluation of an aggression management training program to cope with workplace violence in the healthcare sector. *Research in nursing & health*, *31*(4), 320–328. https://doi.org/10.1002/nur.20260
Rasmussen, C., Holtermann, A., Bay, H., Søgaard, K. & Birk Jørgensen, M. (2015). A multifaceted workplace intervention for low back pain in nurses' aides: a pragmatic stepped wedge cluster randomised controlled trial. *Pain*, *156*(9), 1786–1794. https://doi.org/10.1097/j.pain.0000000000000234
Ojala, B., Nygård, C. H., Huhtala, H. & Nikkari, S. T. (2017). Effects of a nine-month occupational intervention on health-related quality of life. *Scandinavian journal of public health*, *45*(4), 452–458. https://doi.org/10.1177/1403494817695912
Taulaniemi, A., Kankaanpää, M., Tokola, K., Parkkari, J. & Suni, J. H. (2019). Neuromuscular exercise reduces low back pain intensity and improves physical functioning in nursing duties among female healthcare workers; secondary analysis of a randomised controlled trial. *BMC musculoskeletal disorders*, *20*(1), 328. https://doi.org/10.1186/s12891-019-2678-x

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3. Wrong population (n=7)

Rehabilitation therapists (n=1)

Bottino-Bravo, P. & Reid, L. (2009). Innovations in home care rehabilitation services: advancing best practices in care management and professional development. *Home healthcare nurse, 27*(3), 177–183. https://doi.org/10.1097/01.NHH.0000347684.97691.64

University students (n=1)

Chihara, T., Nishijima, U., Moriya, I., Seo, A., Hatano, H. & Yamada, H. (2016). Optimum design of a lumber support belt to reduce the physical workload of the low back. *Journal of Advanced Mechanical Design, Systems, and Manufacturing*, *10*(1), Article 15-00582. https://doi.org/10.1299/jamdsm.2016jamdsm0012

Various health professionals (e.g., social workers, psychologists) who work in home care settings (n=1)

Dogba, M. J., Menear, M., Brière, N., Freitas, A., Emond, J., Stacey, D., & Légaré, F. (2020). Enhancing interprofessionalism in shared decision-making training within homecare settings: a short report. *Journal of interprofessional care*, *34*(1), 143–146. https://doi.org/10.1080/13561820.2019.1623764

Nurses (without specification the occupational field of work) (n=1)

Hodder, J. N., MacKinnon, S. N., Ralhan, A. & Keir, P. J. (2010). Effects of training and experience on patient transfer biomechanics. *International Journal of Industrial Ergonomics*, *40*(3), 282-288. https://doi.org/10.1016/j.ergon.2010.01.007

Employees of outpatient clinic departments (n=1)

Koehne, K. (2015). A New Threat to the Nursing Workforce: Take a Stand!. *Creative nursing*, *21*(4), 234–241. https://doi.org/10.1891/1078-4535.21.4.234

Community nurses who work in hospitals (n=1)

Szeto, G. P. Y., Law, K. Y., Lee, E., Lau, T., Chan, S. Y. & Law, S. W. (2010). Multifaceted ergonomic intervention programme for community nurses: pilot study. *Journal of advanced nursing, 66*(5), 1022-34. https://doi.org/10.1111/j.1365-2648.2009.05255.x.

Family caregiver (n=1)

Yilmaz, C. K., Aşiret, G. D., Çetinkaya, F., OludaĞ, G. & Kapucu, S. (2019). Effect of progressive muscle relaxation on the caregiver burden and level of depression among caregivers of older patients with a stroke: A randomized controlled trial. *Japan journal of nursing science*, *16*(2), 202–211. https://doi.org/10.1111/jjns.12234

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4. Wrong outcome (n=6)

No outcome related to the interventions' impact on employees health reported (n=6)
Bossen, C., Christensen, L. R., Grönvall, E. & Vestergaard, L. S. (2013). CareCoor: augmenting the coordination of cooperative home care work. *International journal of medical informatics*, *82*(5), e189–e199. https://doi.org/10.1016/j.ijmedinf.2012.10.005
Coogle, C. L., Parham, I. A., Jablonski, R. & Rachel, J. A. (2007). The value of geriatric care enhancement training for direct service workers. *Gerontology & geriatrics education*, *28*(2), 109–131. https://doi.org/10.1300/J021v28n02_08
Feldman, P. H., Ryvicker, M., Evans, L. M. & Barrón, Y. (2019). The Homecare Aide Workforce Initiative: Implementation and Outcomes. *Journal of applied gerontology*, *38*(2), 253–276. https://doi.org/10.1177/0733464817707298
Filmer, T. & Herbig, B. (2020). A training intervention for home care nurses in cross-cultural communication: An evaluation study of changes in attitudes, knowledge and behaviour. *Journal of advanced nursing*, *76*(1), 147–162. https://doi.org/10.1111/jan.14133
Fraser, K. D., Sales, A. E., Baylon, M.B., Schalm, C. & Miklavcic, J.J. (2017). Data for improvement and clinical excellence: a report of an interrupted time series trial of feedback in home care. *Implementation science, 12, Article 66*. https:// 10.1186/s13012-017-0600-
Roelofs, P., Bierma-Zeinstra, S., van Poppel, M., van Mechelen, W., Koes, B. & van Tulder, M. (2010). Cost-effectiveness of lumbar supports for home care workers with recurrent low back pain: an economic evaluation alongside a randomized-controlled trial. *Spine, 35*(26), E1619-26. https://doi.org/10.1097/BRS.0b013e3181cf7244

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5. Wrong study design (n=6)

Study protocol (n=3)
Bölenius, K., Lämås, K., Sandman, P. O. & Edvardsson, D. (2017). Effects and meanings of a person-centred and health-promoting intervention in home care services - a study protocol of a non-randomised controlled trial. *BMC geriatrics*, *17*(1), Article 57. https://doi.org/10.1186/s12877-017-0445-0
Indregard, A. R., Knardahl, S., Emberland, J. S., Skare, Ø. & Johannessen, H. A. (2019). Effectiveness of the Labour Inspection Authority's regulatory tools for work environment and employee health: study protocol for a cluster-randomised controlled trial among Norwegian home-care workers. *BMJ open*, *9*(11), Article e031226. https://doi.org/10.1136/bmjopen-2019-031226
Olson, R., Elliot, D., Hess, J., Thompson, S., Luther, K., Wipfli, B., Wright, R. & Mancini Buckmaster, A. (2014). The COMmunity of Practice And Safety Support (COMPASS) Total Worker Health™ study among home care workers: study protocol for a randomized controlled trial. *Trials, 15,* Article 411. https://doi.org/10.1186/1745-6215-15-411

Qualitative study (n=2)
Westerberg, K. & Tafvelin, S. (2015). Changes in commitment to change among leaders in home help services. *Leadership in health services*, *28*(3), 216–227. https://doi.org/10.1108/LHS-04-2014-0038
Mabry, L., Parker, K., Thompson, S., Bettencourt, K., Haque, A., Luther Rhoten, K., Wright, R., Hess, J. & Olson, R. (2018). Protecting workers in the home care industry: workers' experienced job demands, resource gaps, and benefits following a socially supportive intervention. *Home health care services quarterly, 37*(3), 259-276. https://doi.org/10.1080/01621424.2018.1470590

Cross-sectional study (n=1)
Wagner, J. I. & Gregory, D. M. (2015). Spirit at Work (SAW): fostering a healthy RN workplace. *Western journal of nursing research*, *37*(2), 197–216. https://doi.org/10.1177/0193945914521304

6. No full-text available (n=1)

Beaver, D. (2014). Visiting nurse service and home healthcare visitation safety. *Journal of healthcare protection management, 30*(2), 107–116.

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